

COGNITIVE BEHAVIORAL THERAPY (CBT) – SESSION 9

Reviewing Imagery Work & Beginning Relapse Prevention Planning

Fictional Educational Example — Full Detailed

SESSION INFORMATION

Client:	RK (35 y/o Male)	Therapist:	Dr. A.
Session #:	9 of 16	Focus:	Imagery review + Starting relapse prevention

PRE-SESSION THERAPIST NOTES

Client brought the compassionate letter to his younger self and reported it was emotionally moving but helpful. Belief in the old perfectionistic thought remains low (~25%). Today we will review the imagery letter, continue consolidating gains, and begin explicit **relapse prevention planning** as we move into the second half of therapy.

SESSION START & HOMEWORK REVIEW

Therapist: Good morning, RK. How was your week? Let's start with the compassionate letter you wrote to your younger self.

Client: It was surprisingly emotional to write. I told my younger self that he didn't have to be perfect to be loved and that making mistakes is part of being human. Reading it back made the new belief feel more real and compassionate.

Therapist: That's wonderful. These kinds of exercises often help the new belief move from the head to the heart. How did the behavioral experiment go this week?

Client: I tried making a small, visible mistake on purpose in a low-stakes email (a minor typo I didn't correct). No one commented on it negatively. It reinforced that small imperfections don't lead to disaster.

BEGINNING RELAPSE PREVENTION PLANNING

Therapist: Since we're now past the halfway point, I'd like to start preparing you for when therapy ends. One of the most important parts of CBT is having a clear plan to maintain your gains and handle difficult times in the future. This is called **Relapse Prevention Planning**.

Client: That makes sense. I've been worried about what happens after we finish.

Therapist: That's completely normal. Let's start building your personal plan together. The first step is identifying your **Early Warning Signs** — the signals that your anxiety or old thinking patterns might be returning.

Early Warning Signs we identified together:

- Increased time spent worrying during the day
- Returning to over-checking and over-preparing
- Sleep starting to get disturbed again
- Avoiding social interactions or meetings
- Irritability with wife returning

TOP COPING STRATEGIES

Therapist: Next, let's list the strategies that have worked best for you so far. These will be your "go-to" tools when warning signs appear.

Top Coping Tools for RK:

- Use thought records or the balanced belief when the old thought appears
- Practice Worry Time / Worry Postponement
- Do a quick behavioral experiment if a fear feels strong
- Use problem-solving steps instead of worrying
- Schedule one non-work relaxing activity per day
- Talk to wife instead of withdrawing

HOMEWORK ASSIGNMENT

Therapist: For this week:

1. Continue daily diary + Worry Time / Postponement
2. Finalize and write down your personal Early Warning Signs list
3. Create a one-page "Coping Card" with your top 5 strategies (we can format it nicely next session)
4. Continue behavioral experiments and rate belief in the old thought daily

SESSION SUMMARY & FEEDBACK

Therapist: Today we reviewed your excellent progress with the imagery letter and experiments, and we started building your relapse prevention plan by identifying early warning signs and top coping tools. How does that feel?

Client: It feels good to start planning for the future. Having concrete tools written down makes me feel more confident about managing after therapy ends.

Therapist: That's exactly the goal. Any questions?

Client: No, I'm clear.

POST-SESSION THERAPIST NOTES

- Strong engagement with imagery rescripting follow-up work.
- Client showing good motivation for relapse prevention planning.
- Belief in old core thought remains low and stable.
- Next session: Finalize relapse prevention plan (Early Warning Signs + Coping Card), continue experiments, prepare for second half of therapy.

Disclaimer: This is a completely fictional but realistic simulation created solely for educational and AI training purposes. It does not represent any real person or therapy.